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REXOM™ ASCORBIC ACID TABLET 100MG

Name and Strength of Active Ingredients:

Ascorbic Acid100mg

Product Description

Orange, round and top scored tablet.

Pharmacodynamics

Vitamin C (Ascorbic Acid)

Ascorbic acid, coupled with dehydroascorbic acid to which it is reversibly oxidised, has a variety of functions in cellular oxidation processes. Ascorbic acid is required in several important hydroxylation, including the conversion of proline to hydroxyproline (and thus in collagen formation e.g. for intercellular substances and during wound healing); the formation of the neurotransmitters 5-hydroxytryptamine from tryptophan and noradrenaline from dopamine; and the biosynthesis of carnitine from lysine and methionine. Ascorbic acid appears to have an important role in metal ion metabolism, including the gastrointestinal absorption of iron and its transport between plasma and storage organs. There is evidence that ascorbic acid is required for normal leucocyte functions and that it participates in the detoxification of numerous foreign substances by the hepatic microsomal system. Deficiency of ascorbic acid leads to scurvy, which may be manifested by weakness, fatigue, dyspnoea, aching bones, perifollicular hyperkeratosis, petechia and ecchymosis, swelling and bleeding of the gums, hypochromic anaemia and other haematopoietic disorders, together with reduced resistance to infections and impaired wound healing.

Pharmacokinetics

Absorption

Ascorbic acid is well absorbed from the gastrointestinal tract.

Distribution

Ascorbic acid is widely distributed to all tissues. Body stores of ascorbic acid normally are about 1.5g. The concentration is higher in leucocytes and platelets than in erythrocytes and plasma.

Elimination

Ascorbic acid additional to the body's needs, generally amounts above 200mg daily, is rapidly eliminated; unmetabolized ascorbic acid and its inactive metabolic products are chiefly excreted in the urine. The amount of ascorbic acid excreted unchanged in the urine is dose-dependent and may be accompanied by mild diuresis.

Indications

Vitamin C deficiency.

Recommended Dose

ADULT: 100-250 mg once or twice daily.

CHILDREN ABOVE 6 YEARS OLD: 100 mg three times daily for one week followed by 100mg daily until symptoms abate.

Route of Administration

Oral

Contraindications

Known sensitivity to any of the ingredients.

Ascorbic acid should not be given to patients with hyperoxaluria.

Warning and Precautions

Increased intake of ascorbic acid over a prolonged period may result in an increased renal clearance of ascorbic acid, and deficiency may result if the intake is reduced or withdrawn rapidly.

Interactions with Other Medicaments

Ascorbic acid increases the renal excretion of amphetamine. The plasma concentration of ascorbate is decreased

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by smoking and oral contraceptives. Ascorbic acid increases the absorption of iron. Concomitant administration of aspirin and ascorbic acid may interfere with absorption of ascorbic acid. Renal excretion of salicylate is not affected and does not lead to reduced anti-inflammatory effects of aspirin. Concomitant administration of aluminium-containing antacids may increase urinary aluminium elimination. Concurrent administration of antacids and ascorbic acid is not recommended, especially in patients with renal insufficiency. Co-administration with amygdalin (a complementary medicine) can cause cyanide toxicity. Concurrent administration of ascorbic acid with deferoxamine enhances urinary iron excretion. Cases of cardiomyopathy and congestive heart failure have been reported in patients with idiopathic haemochromatosis and thalassemia receiving deferoxamine who were subsequently given ascorbic acid. Ascorbic acid should be used with caution in these patients and cardiac function monitored. Ascorbic acid may interfere with biochemical determinations of creatinine, uric acid and glucose in samples of blood and urine.

Pregnancy & Lactation

Pregnancy

For ascorbic acid no clinical data on exposed pregnancies are available. Animal studies do not indicate direct or harmful effects with respect to pregnancy, embryonal/foetal development, parturition or postnatal development. Pregnant women should exercise caution.

Breast-feeding

Ascorbic acid is excreted in breast milk. Though again caution should be exercised, no evidence exists suggesting such excretion is hazardous to the infant.

Side Effects

Nervous system disorders: headache.

Vascular disorders: flushing.

Gastrointestinal disorders: nausea, vomiting and stomach cramps. Large doses of ascorbic acid may cause diarrhoea.

Skin and subcutaneous tissue disorders: redness of skin.

Renal and urinary disorders: Patients known to be at risk of hyperoxaluria should not ingest ascorbic acid doses exceeding 1g daily as there may be increased urinary oxalate excretion.

Symptoms and Treatment of Overdose

Symptoms

At doses of over 3g per day unabsorbed ascorbic acid is mainly excreted unmetabolised in the faeces. Absorbed ascorbic acid additional to the body's needs is rapidly eliminated. Large doses of ascorbic acid may cause diarrhoea and the formation of renal oxalate calculi. Symptomatic treatment may be required. Ascorbic acid may cause acidosis or haemolytic anaemia in certain individuals with a deficiency of glucose 6-phosphate dehydrogenase. Renal failure can occur with massive ascorbic acid overdosage.

Management

Gastric lavage may be given if ingestion is recent otherwise general supportive measure should be employed as required.

Storage condition

Keep container tightly closed. Store in dry place below 30°C. Protect from light.

Dosage Forms and Packaging Available

100's tablets per bottle

Registration Number:

MAL19911363XZ

Manufactured by & Product Registration Holder:

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